

NHS Devon Integrated Care Board Service Specification

Service Specification No. :	7
Service:	Medical Services for Homeless People
Commissioner Lead:	Lucy Morris, Primary Care Project Officer
Provider Lead:	
Period:	1st April 2024 – 31st March 2029 (3 months' notice will be required to terminate this contract by either party)
Date of Review:	31st March 2025

1. Population Needs (Mandatory)

1.1 National/local context and evidence base

Homelessness is commonly used to describe a wide range of circumstances where people have no secure home. Homelessness is defined in legislation for the purpose of determining entitlement to help from local authorities. Certain groups are defined by law as being in priority need of housing. These include pregnant women, families with children, all 16- and 17-year-olds, those who have physical and mental health problems, people who have experienced domestic or racial violence and people who are vulnerable following a stay in institutions.

However, in order to target health services on the most difficult to engage homeless people, it is necessary to consider a wider range of clients. Many of the most chaotic and vulnerable may not be in contact with housing authorities. Groups to consider are:

- (i) rough sleepers
- (ii) hostel and night shelter residents
- (iii) bed- and-breakfast residents
- (iv) individuals referred by Torbay Council's Homelessness Hospital Discharge Worker

Although the nature and extent of the health problems that face homeless people will vary according to their particular experience of homelessness (for example rough sleepers are often more likely to suffer from musculo-skeletal problems), research has shown that across the board, people who are homeless face an increased risk of psychiatric illness, physical illness, of contracting infectious disease and substance and drink and drug abuse.

There is evidence that medical services for the homeless targeted at a "critical mass" of patients are effective.

2. Outcomes

2.1 NHS Outcomes Framework domains & Indicators

<https://www.wp.dh.gov.uk/publications/files/2012/11/121109-NHS-Outcomes-Framework-2013-14.pdf>

Domain 1	Preventing People from dying prematurely	■ Potential Years of Life Lost from causes amenable to healthcare ■ Reducing premature mortality from the major causes of death ■ Reducing premature death in people with serious mental illness □ Reducing deaths in babies and young children □ Reducing premature death in people with a learning disability
Domain 2	Enhancing quality of life for people with long-term conditions	■ Health related quality of life for people with long term conditions ■ Ensuring people feel supported to manage their condition ■ Improving functional ability in people with long-term conditions □ Reducing time spent in hospital by people with long term conditions ■ Enhancing quality of life for people with mental illness □ Enhancing quality of life for people with dementia
Domain 3	Helping people to recover from episodes of ill health or following injury	■ Emergency admissions for acute conditions that should not usually require hospital admission □ Emergency readmissions within 30 days of discharge from hospital □ Improving outcomes from planned treatments □ Preventing lower respiratory tract infections in children from becoming serious □ Improving recovery from injuries and trauma

		☐ Improving recovery from stroke ☐ Improving recovery from fragility fractures ☐ Helping older people recover their independence after illness or injury
Domain 4	Ensuring people have a positive experience of care	☒ Patient Experience of Primary Care (GP services/GP Out of Hours/NHS Dental Services) ☐ Patient Experience of Hospital Care ☐ Friends and Family Test ☐ Improving people's experience of out-patient care ☐ Improving hospitals' responsiveness to personal needs ☒ Improving access to primary care services ☐ Improving women and families experience of maternity services ☐ Improving the experience of care of people at the end of their lives ☒ Improving experience of healthcare for people with mental illness ☐ Improving children and young people's experience of healthcare ☐ Improving people's experience of integrated care
Domain 5	Treating and caring for people in safe environment and protecting them from avoidable harm	☐ Patient Safety Incidents Reported (safety incidents involving severe harm or death/hospital deaths attributable to problems in care) ☐ Reducing the incidence of avoidable harm ☐ Improving the safety of maternity services ☐ Delivering safe care to children in acute settings

2.2 Locally defined outcomes

- Numbers of clients using the service
- Provision of three 2 hour GP sessions, and one 4 hour nurse sessions at Leonard Stocks
- Positive feedback from various partners (including The Shekinah Mission, Torbay Council)
- Positive patient satisfaction with evidence of Friends and Family Survey
- As required, brief audits of service and case studies
- Appropriate clinical settings
- Robust record-keeping, with IT links to main surgery sites
- Appropriate onward referral to more specialised services as required
- Joint working with other agencies
- Reduced use of secondary care although it is acknowledged that this may be difficult to demonstrate

3. Scope (Mandatory)

3.1 Aims and objectives of service

The aim and objectives of the service are to ensure that:

- Homeless people have equal access to appropriate levels of service from practices designed to ensure that their health needs are effectively tackled
- GPs are provided with the knowledge, training and resources to enable them to deal effectively with homeless people's health needs
- GP services are empowered to tackle the health needs of homeless people holistically by working with relevant services (e.g. housing & social services) to integrate homeless people into local communities.

3.2 Service description/care pathway

This service will fund:

- The development and production of an up-to-date register of patients.
- Liaison with local statutory services and other services working with the homelessness
- The proactive promotion of health services to the local homelessness community ensuring that they are aware of the services available to them
- Flexible registration procedures allowing for permanent registration to anyone who wants it
- Flexible appointment systems including walk in surgeries and longer appointment times for people with multiple needs
- The provision of training to appropriate practice staff ensuring an understanding of and sensitivity towards the particular problems faced by homeless people
- Provision for appropriate and regular screening assessments based on current research in relation to the health needs and problems of homeless people
- The use of relevant guidelines on the prescription of drugs in particular if medication has street value or potential toxicity

- Provision of a health promotion and harm minimisation programme
- Appropriate referral to mental health and drug and alcohol services
- Specialist assessment of the physical and mental health of homeless people when registering including:
 - a high index of suspicion for conditions of TB, hepatitis B and C and HIV and ensuring that screening is made available where appropriate
 - a high index of suspicion of substance use and, where appropriate, initial assessment and/or referral
 - assessment of psychological wellbeing and referral if necessary
- Undertake an annual review which could include feedback from homeless patients, an audit of physical and mental health problems experiences by the homeless, the length and number of consultations for each homeless patient and referrals to and use of other services both within and outside the practice by homeless patients.

3.3 Population covered

Those who are termed “homelessness”, that is those in the position of having no secure home which may include rough sleepers, hostel and shelter residents, and placed B&B residents.

3.4 Any acceptance and exclusion criteria

Those who are homeless, having no secure home.

3.5 Interdependencies with other services

The service has close links with the Shekinah Mission, operating out of the Leonard Stocks Centre on Factory Row, Torquay. The service should liaise with the Torbay Council Homelessness Hospital Discharge Worker.

4. Applicable Service Standards (Mandatory)

4.1 Applicable standards set out in Guidance and/or issued by a competent body (e.g. Royal Colleges)

The Faculty for Homeless Health (2001) has established a set of standards for commissioners and service providers (<http://www.pathway.org.uk/wp-content/uploads/2014/01/Standards-for-commissioners-providers-v2.0-INTERACTIVE.pdf>). We would expect the provider to adhere to the general principles and standards in part 3 and those specific for primary care.

4.2 Applicable local standards

The doctors and nurses who provide the service should be able to satisfy at appraisal and revalidation that they have experience, training and competence to enable them to provide this service.

5. Key Service Outcomes (Mandatory)

- Numbers of clients using the service
- The practice is commissioned to provide the following:

Description	Hours
GP	6
Nurse	6
ACP	2.5
Adhoc GP admin	1
Admin	2.5
Annual Report	10
Management	0.75

- Positive feedback from various partners (including The Shekinah Mission, Torbay Council)
- Positive patient satisfaction as corroborated by suitable survey
- Ensure that patients are not doubly registered
- Ensure that the premises meets CQC standards for outreach surgeries
- Ensure risk assessment has been carried out for lone working with this client group

- Appropriate clinical settings
- Robust record-keeping, with IT links to main surgery sites
- Appropriate onward referral to more specialised services as required
- Joint working with other agencies
- Reduced use of secondary care although it is acknowledged that this may be difficult to demonstrate
- Includes commitment to work with digital team to improve IT on site

6. Applicable quality requirements and CQUIN goals

See note below.

7. Financial Requirements

Expected Contract Values, Details of locally agreed tariff arrangements

£58,109.00 - This figure will be reviewed annually (New annual value would be subject to annual LES uplift in line with all LES's).

This price is all-inclusive of all necessary requirements to provide the service as specified. The prices include the timely provision of activity, quality and performance information including CQUIN if one is developed. Including furnishing of surgery room and consumables as required.

The ICB will be mindful of national guidance relating to any annual inflationary uplift.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- suspension of some or all elements of it,
- redirection of patients to alternative provider(s),
- in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider

8. Information Requirements

An annual report will be required which will include:

- Numbers of clients using the service
- Feedback from various partners (including The Shekinah Mission, Torbay Council)
- Patient satisfaction
- Details of any audits undertaken
- Number and type of referrals made
- Review of ED attendance rates and admission profiles, reporting identified service gaps or commissioning opportunities in line with unplanned DES (this is in addition to the unplanned DES cohort)
- Commentary on the Faculty for Homeless Health outcome standards

9. Location of Provider Premises (Mandatory)

The service will be provided from the centre below;

- Leonard Stocks Centre on Factory Row.

At other times, patients may be seen at the Surgery, Torquay.

10. Individual Service User Placement (Non mandatory)

Not applicable.